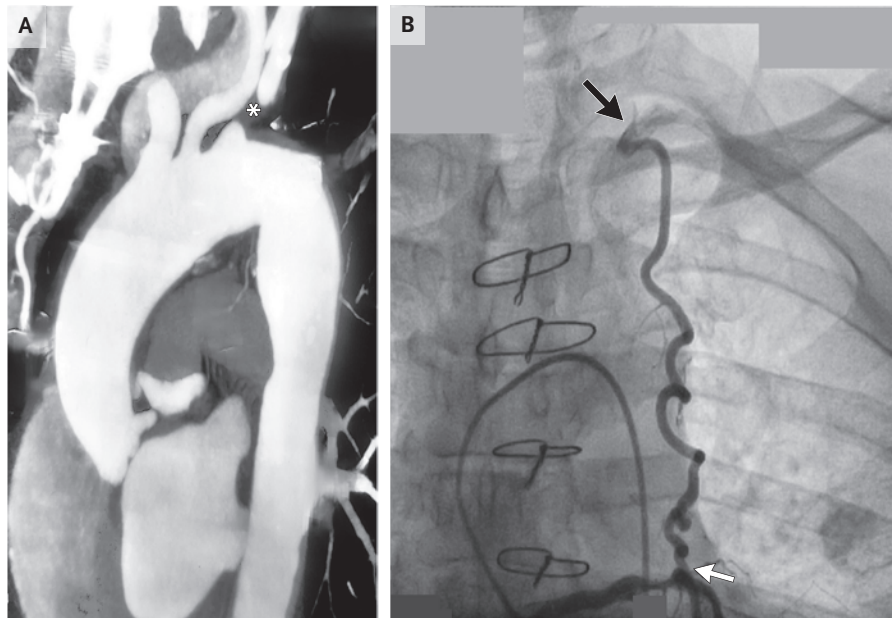


IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

Coronary Subclavian Steal Syndrome



A 63-YEAR-OLD MAN WITH A HISTORY OF CORONARY-ARTERY BYPASS GRAFTING presented to the cardiology clinic with a 2-month history of angina that occurred during exertion of the left arm. Five years before presentation, he had undergone grafting of the left internal thoracic artery (LITA) to the left anterior descending (LAD) coronary artery. The blood pressure was 128/84 mm Hg in the right arm and unmeasurable in the left arm. On physical examination, the brachial and radial pulses on the left side were feeble. A computed tomographic angiogram of the aorta and neck vessels showed total occlusion of the subclavian artery on the left side, proximal to the origin of the LITA (Panel A, asterisk). Coronary angiography with injection of contrast in the left main coronary artery showed retrograde flow in the LITA graft from the LAD coronary artery (Panel B, white arrow) to the left subclavian artery (Panel B, black arrow; see video). A diagnosis of coronary subclavian steal syndrome was made. Coronary subclavian steal syndrome occurs when a clinically significant subclavian-artery stenosis proximal to the origin of the internal thoracic artery graft on the same side leads to a reversal of flow through the internal thoracic artery away from the coronary circulation, resulting in angina during exertion of the arm. After stenting of the left subclavian artery, the patient's angina resolved.

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Mit Chaudhari, M.D.

Vishal Sharma, M.D., D.M.

UN Mehta Institute of Cardiology and
Research Center
Ahmedabad, India
mitchaudhari16@gmail.com



A video is
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